



STROKE PROGRAM CHARTER

Comprehensive Interdisciplinary Stroke Care Program

Aligned with the American Heart Association / American Stroke Association

**Get With The Guidelines®–Stroke Program and American Stroke Association International GL
Stroke Center Certification Standards**

Document Code: SU-001-DOC-01	Version: 1.0
Parent Policy: SU-001 - Stroke Program Organization, Governance and Staffing	Master Manual: SU-MANUAL-01

Charter Period: July 2025 – June 2027

(Reviewed Annually)

Prepared by

Dr. Mohammad Zailaie

Stroke Medical Director

Document Control	
Document Title	Stroke Program Charter
Charter Period	July 2025 – June 2027
Review Cycle	Annual
Document Owner	Stroke Medical Director
Approving Authority	Executive Sponsor / Hospital Executive Committee
Accreditation Reference	AHA/ASA Get With The Guidelines–Stroke; American Stroke Association International GL Stroke Certification Standards
Status	Active

Table of Contents

1. Program Overview
2. Program Vision
3. Program Mission
4. Program Scope of Services
5. Program Goals (2026–2027)
6. Organizational Structure & Governance
7. Interdisciplinary Team Structure
8. Interdisciplinary Relationships
9. Team Member Expectations
10. Program Accountability
11. Stroke Committee Charter
12. Program Performance Indicators (AHA/ASA-Aligned)
13. Data Management & Registry Participation
14. Strategic Accomplishments (Most Recent 12 Months)
15. Strategic Plan (Next 12 Months)
16. Quality Improvement Methodology
17. Education & Competency Requirements
18. Annual Review & Revision History
19. Approval

1. Program Overview

The Stroke Program is an interdisciplinary, hospital-wide program dedicated to delivering evidence-based, timely, and patient-centered stroke care across the full continuum of care — from prevention and prehospital response through acute treatment and rehabilitation.

The program integrates the Emergency Department, Emergency Medical Services (EMS), Neurology, Radiology, Intensive Care Unit (ICU), Neurosurgery, Internal Medicine, Rehabilitation Services, Nursing, Pharmacy, Laboratory, Quality Management, Case Management, and Hospital Administration to achieve optimal, guideline-concordant outcomes for patients with stroke.

This charter is maintained in alignment with American Heart Association / American Stroke Association (AHA/ASA) Get With The Guidelines®—Stroke performance measures and American Stroke Association International GL stroke center certification standards.

2. Program Vision

To become a regional leader in comprehensive stroke care by delivering safe, timely, equitable, and evidence-based treatment that improves patient outcomes and reduces disability across the communities served.

3. Mission

To provide coordinated, high-quality stroke care through:

- Rapid identification and triage of stroke symptoms
- Standardized, evidence-based treatment protocols
- Continuous quality improvement and performance measurement
- Staff and community education
- Community awareness and stroke prevention outreach
- Interdisciplinary collaboration across the continuum of care

4. Program Scope of Services

The Stroke Program encompasses the following clinical conditions and care pathways:

- Acute Ischemic Stroke
- Intracerebral Hemorrhage
- Subarachnoid Hemorrhage
- Transient Ischemic Attack (TIA)
- Cerebral Venous Sinus Thrombosis
- Mechanical Thrombectomy pathway
- Intravenous Thrombolysis pathway
- Dedicated Stroke Unit care
- Neurocritical Care
- Inpatient and Outpatient Rehabilitation
- Secondary Prevention Services

- Community Prevention Programs
- Telestroke Services

5. Program Goals (2026–2027)

5.1 Clinical Goals

Measure	Target Time	Target Compliance
Door-to-CT completion	≤ 20 minutes	≥ 90% of cases
Door-to-Needle (IV thrombolysis)	≤ 60 minutes	≥ 85% of eligible cases
Door-to-Groin puncture (thrombectomy)	Within national benchmark	100% of eligible cases
NIHSS documentation	On arrival	100% of stroke patients
Dysphagia screening	Prior to oral intake	100% of stroke patients
Antithrombotic therapy at discharge	—	≥ 95%
High-intensity statin at discharge	—	≥ 95%
Smoking cessation counseling	—	100% of eligible patients
Stroke education provided	—	100% of patients/caregivers

5.2 Patient Outcome Goals

- Reduce in-hospital mortality
- Reduce symptomatic intracranial hemorrhage (sICH)
- Reduce stroke-related complications
- Reduce average Length of Stay (LOS)
- Improve Modified Rankin Scale (mRS) outcomes at discharge/90 days
- Reduce 30-day all-cause readmissions

5.3 Operational Goals

- Improve Stroke Code activation and response time
- Improve EMS prenotification rates
- Improve CT turnaround time
- Expand Telestroke coverage
- Increase staff competency through simulation and education
- Improve stroke registry data completeness

6. Organizational Structure & Governance

The Stroke Program operates under a defined governance structure that provides clear lines of authority, accountability, and reporting, consistent with AHA/ASA and American Stroke Association International GL stroke center leadership requirements.

6.1 Reporting Structure

- Hospital Executive Leadership
 - Hospital Medical Director (Executive Sponsor)
 - Stroke Medical Director
 - Stroke Program Coordinator
 - Multidisciplinary Stroke Committee

Reporting flows from the Multidisciplinary Stroke Committee upward through the Stroke Program Coordinator, Stroke Medical Director, Hospital Medical Director (Executive Sponsor), to Hospital Executive Leadership.

7. Interdisciplinary Team Structure

Position	Responsibilities	Accountable To
Executive Sponsor	Strategic oversight and resource allocation	Hospital Executive Committee
Stroke Medical Director	Clinical leadership and protocol ownership	Executive Sponsor
Stroke Program Coordinator	Daily program management and coordination	Stroke Medical Director
Emergency Medicine Lead	ED stroke workflow	Stroke Medical Director
Neurology Lead	Stroke diagnosis and treatment	Stroke Medical Director
Radiology Lead	Imaging availability and turnaround	Stroke Medical Director
ICU Lead	Neurocritical care	Stroke Medical Director
Neurosurgery Lead	Surgical management	Stroke Medical Director
Nursing Director	Nursing standards and competency	Executive Sponsor
Rehabilitation Lead	Early rehabilitation	Stroke Medical Director
Pharmacy Lead	Thrombolytic safety and medication management	Stroke Medical Director
Laboratory Lead	Critical laboratory turnaround	Stroke Medical Director
EMS Liaison	Prehospital coordination	Stroke Program Coordinator
Quality Department	KPI monitoring and reporting	Executive Sponsor
Case Management	Discharge planning	Stroke Program Coordinator

8. Interdisciplinary Relationships

Each participating discipline agrees to:

- Participate in Stroke Committee meetings
- Follow approved stroke protocols and order sets
- Participate in quality improvement initiatives
- Share accountability for patient outcomes
- Participate in data review and registry submission
- Participate in annual competency assessment
- Report safety events through the hospital event-reporting system
- Support accreditation and certification survey activities

9. Team Member Expectations

Each Stroke Program team member is expected to:

- Attend at least 80% of Stroke Committee meetings
- Complete annual stroke-specific education
- Maintain competency in stroke protocols
- Participate in mock stroke drills and simulations
- Support timely and accurate data collection
- Participate in Root Cause Analysis (RCA) when required
- Promote a culture of patient safety
- Support continuous performance improvement

10. Program Accountability

10.1 Clinical Excellence

- Evidence-based practice
- Guideline compliance (AHA/ASA)
- Patient safety

10.2 Operational Excellence

- Timely treatment
- Efficient workflow
- Appropriate resource utilization

10.3 Quality

- Continuous monitoring
- Registry participation (Get With The Guidelines–Stroke)
- Audit and certification compliance

10.4 Education

- Staff education
- Community awareness
- EMS collaboration

11. Stroke Committee Charter

11.1 Membership

- Emergency Department (ED)
- Neurology
- Radiology
- Intensive Care Unit (ICU)
- Neurosurgery
- Laboratory
- Pharmacy
- Rehabilitation Services (PT/OT/ST)
- Nursing
- EMS Liaison
- Quality Management
- Case Management / Discharge Planning

11.2 Committee Responsibilities

The Stroke Committee shall:

- Meet monthly
- Review key performance indicators (KPIs)
- Review stroke cases
- Review complications
- Review mortality
- Review thrombolysis cases
- Review thrombectomy cases
- Review treatment and process delays
- Recommend quality improvement actions
- Monitor implementation of approved recommendations

12. Program Performance Indicators (AHA/ASA-Aligned)

Performance indicators are tracked in alignment with Get With The Guidelines–Stroke measure specifications and reported to the Stroke Committee and hospital leadership.

12.1 Clinical Indicators

- Door-to-CT
- Door-to-Needle
- Door-to-Puncture
- NIHSS completion
- Dysphagia screening
- VTE prophylaxis
- Antithrombotic therapy at discharge

12.2 Operational Indicators

- Stroke Code activation time
- CT reporting time
- EMS prenotification rate
- Stroke alert volume
- Interfacility transfer times

12.3 Outcome Indicators

- In-hospital mortality
- Symptomatic ICH
- Length of Stay (LOS)
- 30-day readmission rate
- Modified Rankin Scale (mRS)
- Patient satisfaction

13. Data Management & Registry Participation

The Stroke Program maintains active participation in the American Heart Association / American Stroke Association Get With The Guidelines®–Stroke registry to support standardized data abstraction, benchmarking, and continuous performance measurement.

- Consecutive case capture of all stroke and TIA admissions
- Timely data abstraction and registry submission
- Internal data validation and inter-rater reliability checks
- Quarterly benchmarking against regional and national performance
- Registry data used to drive Stroke Committee quality improvement priorities

14. Strategic Accomplishments (Most Recent 12 Months)

The Stroke Program successfully achieved:

- Monthly interdisciplinary Stroke Committee meetings
- Continuous monitoring of stroke quality indicators
- Implementation of standardized, evidence-based stroke pathways
- Regular multidisciplinary case reviews
- Stroke simulation drills for emergency response teams
- Staff education on acute stroke management
- Improvement initiatives targeting Door-to-CT and Door-to-Needle times
- Enhanced collaboration with EMS for prenotification and rapid activation
- Expansion of quality data collection through the stroke registry
- Increased compliance with evidence-based discharge measures
- Strengthened patient and family education on stroke prevention and recovery

15. Strategic Plan (Next 12 Months)

Objective 1 — Improve Door-to-Needle Performance

- Lean workflow redesign
- Monthly audits
- Real-time feedback to care teams
- Simulation training

Objective 2 — Improve Mechanical Thrombectomy Pathway

- Faster CTA workflow
- Direct angiography-suite activation
- EMS education

Objective 3 — Strengthen the Stroke Unit

- Nursing competency development
- Early rehabilitation initiation
- Daily multidisciplinary rounds

Objective 4 — Enhance Community Prevention

- Public awareness campaigns
- Community stroke screening
- FAST (Face, Arm, Speech, Time) public education

Objective 5 — Expand Telestroke Services

- Onboard additional partner hospitals
- Standardize teleconsultation workflow

- KPI monitoring for telestroke encounters

Objective 6 — Accreditation Readiness

- Conduct mock certification surveys
- Comprehensive document review
- Performance audits
- Staff interviews and readiness assessment

16. Quality Improvement Methodology

The Stroke Program utilizes the Plan–Do–Study–Act (PDSA) methodology to:

- Identify performance gaps
- Implement targeted interventions
- Measure outcomes
- Sustain improvements

Performance data are reviewed monthly by the Stroke Committee and reported quarterly to hospital leadership, consistent with American Stroke Association International GL performance improvement requirements for certified stroke centers.

17. Education & Competency Requirements

Consistent with AHA/ASA and stroke certification standards for staff competency, the program requires:

- Annual stroke-specific education for all core stroke team members
- NIHSS certification/re-certification for designated clinical staff
- Participation in mock stroke code drills
- EMS education on prenotification and stroke recognition
- Community and patient/caregiver education on stroke prevention, recognition, and recovery

18. Annual Review & Revision History

This Program Charter shall be reviewed annually by:

- Stroke Medical Director
- Stroke Program Coordinator
- Stroke Committee
- Executive Leadership

Revisions will be approved by the Executive Sponsor.

Revision History

Version	Date	Summary of Changes	Approved By
1.0	July 2025	Initial charter issued	Executive Sponsor

19. Approval

Role	Name	Signature	Date
Executive Sponsor			
Stroke Medical Director			
Stroke Program Coordinator			
Nursing Director			
Quality Director			